Prescription & Medication Coordination - Deep Dive Report

For Alex Rivera · About Ari Rivera · 2026-07-31

This report reflects one parent's perspective. It does not diagnose, provide medical advice, prescribe, or determine parental fitness. It is intended to support coordination and to prompt conversations with the prescribing provider, pharmacist, school nurse, mediator, or family professional. In a medical emergency call 911 or the poison control centre.

1. Medication Profile

Reported medication categories

- Daily maintenance medication
- Rescue / emergency medication (e.g., inhaler, EpiPen, glucagon)
- Controlled substance (e.g., stimulant, benzodiazepine, opioid)

Prescribers involved

- Paediatrician
- Psychiatrist

Controlled substance: Yes · School medication: Yes

Additional notes

Ari uses a daily inhaler plus a stimulant for ADHD.

At a glance

10 sections completed. 5 area(s) flagged. 1 area(s) recognised as strengths.

Safety-relevant signals

- Medication Supply Between Homes: Rescue / emergency medication is available in only one home.
- Controlled Medication Accountability: Only 2 of the controlled-medication safeguards are in place across both homes.

Sections to follow up on

- Prescription Pick-Up
- Medication Supply Between Homes
- Financial & Insurance Barriers
- Controlled Medication Accountability
- School Medication Coordination

Areas of current strength

- Medication Decisions & Disagreements

2. Prescription Pick-Up

Area to develop

Prescription pick-ups cause friction, delays, or missed doses.

Signals: Average of 2 responses is 2.5.

Recommendations

- Decide in writing who picks up each recurring prescription and who is the backup.
- Send a short pick-up confirmation to the other parent on the day it is filled.
- Enable pharmacy auto-refill or delivery to reduce human error.

3. Medication Supply Between Homes

Area to develop

Supply between homes is inconsistent, and the child has risked running out.

Safety: Rescue / emergency medication is available in only one home.

Recommendations

- Keep a labelled backup supply of daily medication in each home.
- Confirm rescue / emergency medication (inhaler, EpiPen, glucagon) is present, unexpired, and accessible in BOTH homes.
- Set a shared calendar reminder five days before every refill is due.
- Check expiration dates on the first of each month.

7. Financial & Insurance Barriers

Area to develop

Financial or insurance issues could interrupt medication.

Signals: 1 financial or insurance barrier(s) currently affecting medication.

Recommendations

- Document the medication cost-split in the parenting plan or a signed side letter.
- Verify insurance formulary coverage before any new prescription.
- Ask the prescriber about therapeutic generics or manufacturer discount programs.
- For prior authorisations, agree in writing which parent submits and follows up.

8. Controlled Medication Accountability

Area to develop

Controlled-medication accountability needs strengthening.

Safety: Only 2 of the controlled-medication safeguards are in place across both homes.

Recommendations

- Store controlled medication in a locked location in both homes.
- Track pill / dose count in writing at every handoff and sign it.
- Ask the prescriber for a controlled-substance agreement signed by both parents.
- Only the parent should administer doses; the child should never have access to the bottle.

9. School Medication Coordination

Area to develop

School medication process has gaps that could delay a dose.

Signals: Only 3 of the school medication elements are in place.

Recommendations

- Ensure both parents are named on the school medication authorisation and emergency plan.
- Ask the school nurse to send the Medication Administration Record (MAR) to both parents each month.
- Pre-authorise field-trip and off-campus medication once per school year.
- Notify the school within 24 hours of any dose change.

10. Medication Decisions & Disagreements

Strength

Most medication decisions consistently see agreement.

Signals: 5 of 8 medication decision areas consistently see agreement.

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